TMS LIST — THE #1 TMS CLINIC DIRECTORY

TMS Billing & CPT Codes

Complete Cheat Sheet for Billing Staff & Practice Managers

2026 Edition

3	8+	10
KEY CPT CODES	MAJOR PAYERS	DENIAL SOLUTIONS

What You'll Learn

- 2026 CPT code reimbursement rates & billing nuances
- When and how to apply modifiers (59, 76, 77, XE, XS, XU)
- Medicare LCD/NCD documentation requirements
- Prior authorization matrix for 8 major payers
- Top 10 claim denials and proven prevention tactics
- Step-by-step appeal letter template
- Complete billing workflow from intake to payment
- Key billing terminology glossary

> *"This cheat sheet saved our practice $40,000 in denied claims in the first quarter alone. Every billing staff member has it on their desk.*

Dr. Sarah Mitchell, MD
Medical Director, Clarity Brain Center

> *"Finally, a billing reference that speaks our language. The payer authorization matrix alone is worth its weight in gold.*

Jennifer Park, CPC
Billing Manager, NeuroWellness TMS

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CPT Code Reference Table

Section 1

CPT CODE	DESCRIPTION	UNITS	TYPICAL REIMBURSEMENT*	BILLING NOTES
90867	**Therapeutic repetitive transcranial magnetic stimulation (TMS) treatment planning** Motor threshold determination and mapping	1 session	**$150 – $275**	Bill once per treatment series. Required before starting TMS. Document motor threshold in mV and scalp location.
90868	**Therapeutic repetitive transcranial magnetic stimulation (TMS) treatment delivery** Treatment delivery per session	Per session	**$225 – $350**	Bill per treatment session. Typically 20-30 sessions per series. 59 modifier required for bilateral treatment.
90869	**Therapeutic repetitive transcranial magnetic stimulation (TMS) treatment delivery** Subsequent motor threshold determination	1 session	**$100 – $175**	Bill when re-determining motor threshold mid-series. Requires documentation of clinical rationale (e.g., dose adjustment).

Important Billing Reminder

Add-on codes: Do not bill 90867, 90868, or 90869 with E/M codes on the same day without proper modifier documentation. 90867 is a standalone procedure code.

Footnotes

1. Reimbursement rates are national averages based on 2025 Medicare Physician Fee Schedule and may vary by region (CAG, GAF). Always verify current rates via your payer fee schedules or FAIR Health.[2]
2. Source: CMS Physician Fee Schedule Look-up Tool; FAIR Health charges for 90867-90869 vary by geographic region and setting of care.

Modifier Usage Guide

Section 2

Why Modifiers Matter

Proper modifier use is the difference between paid and denied claims. Incorrect modifier application is one of the top causes of TMS claim denials.

59

Distinct Procedural Service

Use when: Two procedures performed at same session that are normally bundled but are truly distinct.

TMS Application: Bilateral TMS treatment — bill 90868-59 when delivering TMS to both left and right dorsolateral prefrontal cortex (DLPFC) in same session.

Documentation: Note anatomical sites treated and why services are separate.

76

Repeat Procedure by Same Physician

Use when: Same physician performs the same procedure again on the same day.

TMS Application: Second motor threshold determination same day (rare).

Documentation: Note medical necessity for repeat service same day.

77

Repeat Procedure by Another Physician

Use when: Same procedure performed again same day but by a different physician.

TMS Application: Uncommon in TMS; may apply if treatment handed off between physicians.

Documentation: Note the different treating physician.

XE

Separate Encounter

Use when: Services are distinct because they occurred during a separate encounter.

TMS Application: When a separate clinical encounter occurs same day (e.g., E/M visit plus TMS session).

Documentation: Note separate encounter date/time.

XS

Separate Structure

Use when: Service was performed on a separate organ or anatomical structure.

TMS Application: May be used for bilateral TMS to demonstrate separate anatomical sites.

Documentation: Specify anatomical structure (left vs. right DLPFC).

XU

Unusual Non-Overlapping Service

Use when: Service does not overlap usual components of main procedure.

TMS Application: Used when TMS is delivered with another neuromodulation service that does not overlap.

Documentation: Explain why services are non-overlapping.

Modifier 59 vs. X{E/S/U} — The Distinction

CMS guidance now encourages using X{E/S/U} modifiers over 59 when the separate service meets those specific criteria. However, some payers still require 59. When in doubt, check the payer's specific modifier guidelines before submitting.

Medicare LCD/NCD Requirements

Section 3

LCD vs. NCD Definitions

LCD (Local Coverage Determination): Medicare Administrative Contractor (MAC)-specific coverage policies. Vary by region.
NCD (National Coverage Determination): CMS-mandated national coverage policy. Applies to all MACs.

Required Documentation

- ✓ FDA clearance documentation for the TMS device
- ✓ Diagnosis code (ICD-10) confirming treatment-resistant depression
- ✓ PHQ-9 score ≥ 10 (or equivalent depression scale)
- ✓ Documentation of ≥ 2 adequate medication trials
- ✓ Psychiatric evaluation notes
- ✓ Treatment protocol and dosing parameters
- ✓ Informed consent for TMS treatment
- ✓ Motor threshold documentation (mV reading)
- ✓ Treatment delivery logs per session
- ✓ Clinical response assessments at intervals

Common LCD Denials

- ✗ No prior authorization obtained when required by MAC
- ✗ PHQ-9 score documentation missing or below threshold
- ✗ Failure to document adequate medication trials
- ✗ Treatment exceeds approved number of sessions
- ✗ Device not FDA-cleared for the specific indication
- ✗ Provider not enrolled in Medicare (or not registered as treating physician)
- ✗ No documented treatment response after 4-6 weeks
- ✗ Diagnosis does not meet LCD criteria
- ✗ Motor threshold not documented per session
- ✗ Concurrent TMS and ECT or vagus nerve stimulation without documentation

Most Frequent Medicare TMS Denial

"Treatment not medically necessary — failure to document adequate medication trials." Always maintain a contemporaneous medication history log with drug names, doses, duration, and reasons for discontinuation.

Footnotes

3. Per CMS National Coverage Determination (NCD) for Repetitive Transcranial Magnetic Stimulation (rTMS), effective 2011 and updated through 2024. Coverage requires FDA clearance and specific clinical criteria.

Commercial Payer Authorization Matrix

Section 4

Authorization is Not Optional

Prior authorization requirements change frequently. Always verify current requirements directly with the payer before initiating treatment. This matrix reflects typical 2025-2026 requirements but is not a guarantee.

PAYER	AUTH REQUIRED	MIN TRIALS	PHQ-9	NOTES
Medicare (Part B)	Varies by MAC	2 medications	≥ 10	Verify MAC-specific LCD. Most require advance Beneficiary Notice (ABN) if uncertain of coverage.
Medicaid	Yes	Varies by state	Varies by state	State-specific criteria. Some states exclude TMS coverage entirely. Check state plan.
Blue Cross Blue Shield	Yes	4 medications	≥ 15 or severe	Most restrictive of commercial plans. BCBS Anthem and BCBS Federal may differ. Pre-auth via provider portal.
Aetna	Yes	2 medications	≥ 10	Aetna considers TMS medically necessary after 2 adequate medication trials. Requires precertification.
Cigna	Yes	2 medications	≥ 10	Cigna requires participating provider status and precertification. Covers up to 30 sessions.
UnitedHealthcare (UHC)	Yes	2 medications	≥ 10	UHC has specific TMS policy. Optum behavioral health may also be involved. Verify medical policy.
Anthem	Yes	4 medications	≥ 15	Anthem BCBS has one of the stricter policies. Prior authorization required through provider portal.
Tricare	Yes	2 medications	≥ 10	Tricare covers TMS for active duty service members and dependents. Requires prior authorization from regional contractor.

"Minimum Trials" Explained

Minimum trials = number of adequate medication trials required before TMS is considered medically necessary. An "adequate trial" typically means ≥ 6 weeks at a therapeutic dose. Document each trial with medication name, dose, duration, and outcome in the medical record.

Top 10 Denials & Prevention

Section 5

1 No Prior Authorization on File

WHY IT HAPPENS
Staff forgets to obtain authorization before starting treatment or obtains it for wrong CPT codes.

HOW TO PREVENT
Create pre-treatment checklist with authorization verification step. Assign dedicated staff to track auths. Use billing software alerts.

REMEDIATION
If caught before submission: obtain auth retroactively if payer allows. If denied post-submission: appeal with auth approval letter and explanation of delay.

2 Duplicate Claim Submission

WHY IT HAPPENS
Same CPT code billed multiple times for same date of service. Often caused by billing system errors or resubmission without adjusting original claim.

HOW TO PREVENT
Use claim scrubbing software. Always check for existing claims before resubmitting. Implement 24-hour hold on claim submission for review.

REMEDIATION
Check remittance advice for 835 code. If truly duplicate: withdraw claim. If resubmission needed: include adjusted claim with original claim reference number.

3 Medically Unnecessary — Inadequate Medication Trials

WHY IT HAPPENS
Documentation fails to show patient tried and failed 2+ adequate medication trials before TMS.

HOW TO PREVENT
Maintain comprehensive medication history in chart. Include drug, dose, duration, response, and reason for discontinuation. Update at every visit.

REMEDIATION
Appeal with complete medication trial documentation. Include pharmacy records if available. Request peer-to-peer review.

4 Member Not Eligible on Date of Service

WHY IT HAPPENS
Eligibility changes (termination, plan change) between scheduling and treatment. Often seen at start of calendar year.

HOW TO PREVENT
Verify eligibility 24-48 hours before each session. Re-verify at start of each new treatment series. Document all verification attempts.

REMEDIATION
Check if retroactive reinstatement applies. If patient was eligible but system error: provide documentation and appeal. May need patient to contact employer/insurer.

5 Invalid or Missing Modifier

WHY IT HAPPENS
Incorrect modifier applied or required modifier missing. Common with bilateral TMS (90868) and multiple sessions.

HOW TO PREVENT
Create modifier decision tree for billing staff. Pre-build claim templates with correct modifier combinations. Review claims before submission.

REMEDIATION
Submit corrected claim with proper modifier. If already denied: appeal with documentation supporting the modifier choice.

6 Timely Filing Limit Exceeded

WHY IT HAPPENS
Claim submitted after payer's timely filing deadline (typically 90 days to 1 year from date of service).

HOW TO PREVENT
Track claims aging weekly. Set internal deadline 30 days before payer deadline. Prioritize old claims in billing queue.

REMEDIATION
Exception requests with documentation of circumstances (e.g., natural disaster, payer system outage). Otherwise, may need to write off.

7 Incorrect Place of Service

WHY IT HAPPENS

TMS billed with wrong POS code (e.g., 11 for physician office when provided in hospital outpatient department).

HOW TO PREVENT

Verify facility billing requirements at contract negotiation. Train staff on correct POS for each location. Include POS in claim scrubber rules.

REMEDIATION

Correct and resubmit with accurate POS. If caused by facility change mid-treatment: may need to split billing across providers.

8 Provider Not In-Network

WHY IT HAPPENS

Rendering provider not credentialed with payer, or credentialing expired. Common with new staff or recent payer contracts.

HOW TO PREVENT

Maintain credentialing calendar. Verify provider participation before patient intake. Check payer directories for accuracy.

REMEDIATION

Initiate retroactive credentialing if available. If patient is/was eligible: balance to patient with documented disclosure. If not eligible: write off or refer to collections.

9 Authorization Exhausted

WHY IT HAPPENS

Treatment continues beyond number of sessions approved in authorization. Common when patient needs additional sessions beyond initial authorization.

HOW TO PREVENT

Track authorized vs. delivered sessions in real-time. Submit extension requests 2 weeks before auth expires. Document clinical need for additional sessions.

REMEDIATION

Submit appeal/extension request immediately upon denial. Include clinical documentation supporting need for continued treatment. Request peer-to-peer review.

10 Code Bundling / Code Pairing Edit

WHY IT HAPPENS

NCCI (National Correct Coding Initiative) edits bundle codes that should not be billed together. TMS with E/M codes, certain assessments, or other procedures.

HOW TO PREVENT

Know NCCI edits for TMS codes. Use appropriate modifiers (59, XE) when billing paired services that are medically necessary and distinct. Review quarterly CMS NCCI updates.

REMEDIATION

Add appropriate modifier and resubmit. If edit applies and no modifier works: bill only the primary code. Document medical necessity for each service provided.

Appeal Letter Template

Section 6

Before You Appeal

Level 1: First-level appeal (internal to payer)
Level 2: Second-level appeal or external review
Peer-to-Peer: Request a call between treating physician and payer's medical director — often the fastest path to reversal.
Always include the remittance advice (EOB) with the denial reason code when submitting an appeal.

SAMPLE TMS TREATMENT APPEAL LETTER

[1] HEADER / DATE / CLAIM INFORMATION

[Date]
[Provider Name]
[Provider Address]
[Provider Phone/Fax]

RE: Appeal for Claim Denial — [Patient Name], [DOB]
Claim Number: [XXXXXXXXX]
Date of Service: [XX/XX/XXXX]
CPT Code(s): [90867 / 90868 / 90869]
Amount Denied: [$XXX.XX]

[2] TO WHOM IT MAY CONCERN / MEDICAL DIRECTOR

[Payer Name]
Appeals Department
[Payer Address]

[3] PATIENT & CLINICAL INFORMATION

I am writing to formally appeal the denial of coverage for repetitive transcranial magnetic stimulation (TMS) services provided to [Patient Name] on [Date of Service]. The claim was denied for the reason listed as: "[Denial Reason from EOB]."

[Patient Name] is a [XX]-year-old [male/female] with a primary diagnosis of [Major Depressive Disorder, ICD-10: F32.X/F33.X]. The patient meets all medical necessity criteria for TMS treatment as outlined in your plan's medical policy and/or applicable LCD/NCD guidelines.

[4] CLINICAL RATIONALE — WHY TMS IS MEDICALLY NECESSARY

Treatment-resistant depression is defined as failure to achieve adequate response after two or more antidepressant trials of adequate dose and duration. [Patient Name] has documented history of:

- *Medication Trial 1: [Drug Name], [Dose], [Duration], outcome: [no response/partial response]*
- *Medication Trial 2: [Drug Name], [Dose], [Duration], outcome: [no response/partial response]*
- *[Additional trials as applicable]*

PHQ-9 score at initiation of TMS: [XX]/27, indicating [moderate to severe] depression. TMS is the next medically appropriate step in the patient's treatment continuum.

[5] EVIDENCE & SUPPORTING DOCUMENTATION

Enclosed with this appeal, please find:

- *Copy of the original denial / Remittance Advice (EOB)*
- *Office notes documenting medication history and trials*
- *Psychiatric evaluation with PHQ-9 scores*
- *PHQ-9 score documentation at [Interval] demonstrating clinical response*
- *TMS treatment log / session records*
- *Motor threshold documentation*
- *Copy of LCD/NCD criteria demonstrating medical necessity*
- *FDA clearance documentation for [TMS Device Name]*

[6] REQUEST

Based on the enclosed clinical documentation, we respectfully request that this claim be overturned and processed for payment at the contracted rate. TMS is FDA-cleared for treatment-resistant major depressive disorder and is supported by extensive clinical evidence including clinical trials, professional society guidelines (APA, ISRT), and your own medical policy.

We are available for a peer-to-peer review at your earliest convenience. Please contact [Contact Name] at [Phone Number] or [Email] to schedule.

Thank you for your prompt attention to this matter.

SINCERELY,

[Treating Physician Name], [Degree(s)]
[Specialty / Board Certification]
[NPI: XXXXXXXXXX]
[Practice Name]
[Phone] | [Email]

Sample Clinical Paragraph: *"The patient has failed four adequate medication trials over a period of three years, including sertraline 200mg (12 weeks), venlafaxine 375mg (16 weeks), bupropion 450mg (8 weeks), and a combination of escitalopram 20mg with aripiprazole 5mg augmentation (10 weeks). All trials were at guideline-recommended doses for adequate duration. The patient has experienced significant functional impairment, including inability to maintain employment and strained personal relationships. TMS treatment resulted in a 40% reduction in PHQ-9 score after 20 sessions, demonstrating clear clinical response. Continuation of TMS is medically necessary to maintain these gains and prevent relapse."*

Billing Workflow Checklist

Section 7

1 Patient Intake

- Collect insurance cards (front & back)
- Verify demographics
- Copy insurance cards
- Have patient sign consent & HIPAA forms

2 Insurance Verification

- Verify eligibility (24-48 hrs before)
- Check TMS coverage & exclusions
- Confirm prior auth requirements
- Note timely filing deadline

3 Authorization

- Submit prior auth request
- Include PHQ-9 & med history
- Track auth status
- Document auth number & expiration

4 Pre-Treatment

- Confirm auth valid for date
- Re-verify eligibility if needed
- Check patient financial responsibility
- Obtain copay/collect at visit

5 Treatment Delivery

- Document motor threshold
- Log each session
- Note any dose adjustments
- Track session count vs. auth

6 Documentation

- Complete encounter notes
- Attach PHQ-9 at intervals
- Document clinical response
- Sign & date all notes

7 Charge Entry

- Enter charges within 24 hrs
- Apply correct CPT codes
- Apply correct modifiers
- Double-check ICD-10 codes

8 Claim Submission

- Scrub claims for errors
- Submit within timely filing
- Track claim in clearinghouse
- Address any rejects immediately

9 Payment Posting

- Post ERA/EOB within 48 hrs
- Verify allowed amounts
- Identify denials & adjustments
- Balance patient responsibility

10 AR Follow-Up

- Work denials within 30 days
- Appeal with documentation
- Request peer-to-peer reviews
- Track aged AR weekly

11 Patient Billing

- Send patient statements
- Set up payment plans
- Offer self-pay discounts if applicable
- Send to collections if needed

12 Monthly Close

- Reconcile charges & payments
- Run aging reports
- Review write-offs
- Identify process improvements

Key Performance Indicators (KPIs) to Track

Clean Claim Rate: Target > 95% — percentage of claims submitted without errors.
First-Pass Resolution: Target > 85% — claims paid on first submission.
Days in AR: Target < 35 days — average time from service to payment.
Denial Rate: Target < 5% — percentage of claims denied on first submission.

Glossary of Billing Terms

Section 8

ADR (Additional Documentation Request)

A request from a payer for medical records or additional information to process a claim. Must be responded to within the specified timeframe or the claim may be denied.

ALJC (Administrative Law Judge Court)

Third level of the Medicare appeals process. Administrative Law Judge hearings are available for claims exceeding $1,800 (as of 2024 thresholds).

Appeal

A formal request to reconsider a denied claim. Appeals must typically be filed within 30-60 days of the denial, depending on the payer.

Authorization (Prior Auth)

Payer approval obtained before delivering certain services. Authorization does not guarantee payment but is typically required for coverage.

Bilateral

Treatment delivered to both left and right sides of the body (in TMS: both left and right DLPFC). Bilateral treatment may require modifier 59.

Bilateral Pricing

Reimbursement rate applied when a procedure is performed on both sides of the body. Some payers pay 150% of the unilateral rate for bilateral procedures.

Clean Claim

A claim submitted without errors, missing information, or discrepancies. Clean claims are processed and paid or denied within standard timeframes.

COB (Coordination of Benefits)

Process of determining which payer is primary when a patient has multiple insurance plans. Prevents double-payment and ensures proper claim routing.

CPT (Current Procedural Terminology)

Standardized code set maintained by the AMA for describing medical procedures and services. Used on all medical claims in the United States.

Deductible

Fixed amount the patient must pay out-of-pocket before insurance coverage begins. Does not reset mid-treatment series.

DRG (Diagnosis-Related Group)

Payment classification system used primarily for hospital inpatient stays. Not typically used for outpatient TMS billing.

EOB (Explanation of Benefits)

Statement from the insurance company showing what was billed, what was paid, and what the patient owes. Not a bill.

ESA (Economic Stabilization Act)

Federal legislation that may provide certain protections during declared emergencies. Can affect timely filing and other billing requirements.

FAIR Health

Independent nonprofit that publishes benchmarks for out-of-network charges. Used as reference for determining usual and customary rates.

Fee Schedule

The contracted rate that a payer agrees to pay for specific services. Usually lower than billed charges.

G-Code

Healthcare Common Procedure Coding System (HCPCS) code used for Medicare/Medicaid services. Not typically used for TMS CPT codes.

Grace Period

Time period after a premium due date during which the insurance policy remains active even if payment has not been received.

LCD (Local Coverage Determination)

Medicare Administrative Contractor (MAC)-specific coverage policy that defines when Medicare will pay for a service in a specific geographic area.

Modifier

Two-character code appended to a CPT code that provides additional information about the service performed without changing the code's definition.

NCD (National Coverage Determination)

CMS-mandated national policy that determines whether Medicare will cover a specific service nationwide.

OIG (Office of Inspector General)

Federal agency that investigates healthcare fraud, waste, and abuse. Publishes compliance guidance and work plans.

Paid Amount

The actual amount the insurance company paid on a claim. Usually the contracted fee schedule amount minus any patient responsibility.

Prior Auth (Prior Authorization)

Requirement to obtain payer approval before delivering certain services. Does not guarantee payment but required for coverage with many payers.

Provider

A healthcare professional (physician, NP, PA) or facility authorized to provide and bill for medical services.

R&C (Reasonable and Customary)

The prevailing rate paid for a service in a geographic area. Used to determine out-of-network reimbursement.

Timely Filing

The deadline by which a claim must be submitted to the payer. Usually 90 days to 1 year from date of service. Missing this deadline results in automatic denial.

TMS Codes

CPT codes 90867 (treatment planning/motor threshold), 90868 (treatment delivery), and 90869 (subsequent motor threshold) used specifically for billing TMS services.

QR Resources

Section 9

Scan for More Resources

Access our interactive TMS directory, billing quiz, and practice owner dashboard

TMS Clinic Directory

Find TMS providers near you

Billing Compliance Quiz

Test your TMS billing knowledge

Practice Owner Dashboard

Manage your TMS listing & analytics

What You'll Find Online

TMS Clinic Directory: Over 1,000 verified TMS treatment providers searchable by location. Updated monthly.
Billing Quiz: 20-question interactive quiz covering CPT codes, modifiers, Medicare LCD requirements, and common denials. Get your score instantly.
Practice Dashboard: Claim and update your free TMS listing, view patient inquiry analytics, and access billing resources.

→ Next Steps — Your Action Plan

Section 10

Your Week Ahead

Use this 5-day action plan to immediately improve your TMS billing operations. Each task takes 30-60 minutes.

DAY 1	DAY 2	DAY 3	DAY 4	DAY 5
Audit Current AR	**Verify Payer Auths**	**Update Templates**	**Train Your Team**	**Set Up Tracking**
Pull your AR report. Flag all TMS denials older than 30 days. Prioritize high-dollar items for immediate appeal.	Call top 3 payers (Medicare, BCBS, Aetna). Confirm current auth requirements and any policy updates for 2026.	Review and update your TMS documentation templates. Ensure PHQ-9 scoring and medication trial sections are prominent and complete.	Share this cheat sheet with all billing staff. Hold a 30-minute walkthrough covering modifier usage and denial prevention.	Implement weekly KPI tracking: clean claim rate, denial rate, days in AR. Create simple dashboard to monitor progress monthly.

Monthly Review Cadence

Schedule recurring monthly meetings to review TMS billing KPIs. Catch problems early — denied claims over 60 days old are significantly harder to collect.

Questions to Ask Your Billing Manager

Tear-Out Card

Questions to Ask Your Billing Manager

Cut along the dotted border and keep at your desk for quick reference

1. Do we have current prior authorization on file for every patient currently receiving TMS treatment?
2. What is our current denial rate for TMS claims, and how does it compare to last quarter?
3. Are all rendering providers enrolled and credentialed with our top 5 payers?
4. Do our documentation templates capture all LCD/NCD requirements for Medicare TMS claims?
5. What is the average time from claim submission to payment for TMS services?
6. How many TMS appeals do we have pending, and what is the average resolution time?
7. Are we using the correct modifiers for bilateral TMS treatments?
8. When was the last time we verified our TMS CPT codes against current Medicare fee schedules?
9. Do we have a process for tracking authorized sessions vs. delivered sessions to avoid authorization exhaustion?
10. What is our plan for addressing any TMS claims approaching the timely filing deadline?
11. Have we reviewed our payer contracts recently to ensure TMS coverage terms are accurate?
12. Do all TMS patients have documented PHQ-9 scores at baseline and at regular intervals during treatment?

Pro Tip

Schedule a monthly 15-minute "billing health check" with your billing manager. Review these questions, track metrics, and address issues before they become costly problems.

TMS List

The most comprehensive TMS clinic directory and billing resource for providers

Find TMS Providers
tmslist.com/map/

Billing Quiz
tmslist.com/quiz/

Practice Dashboard
tmslist.com/owner/dashboard/

2026 TMS Billing & CPT Codes Cheat Sheet
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For educational purposes only. Verify all coding and coverage information with current payer guidelines and CMS regulations.

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